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Hiring Manager

Health Technology and Systems Team
Canada's Drug Agency (CDA-AMC)

Re: Clinical Research Assistant (Temporary)

Dear Hiring Manager,

I'm applying for the Clinical Research Assistant role because the day-to-day work, screening literature, extracting data from studies, drafting reports, and contributing to committee outputs, is the same shape of work I'm already doing in two other research environments. I want to apply those habits to health technology assessment instead of water quality and federal case management.

At Stelis Environmental Solutions, the work isn't "regulatory compliance" in the abstract. It's reading dense source material (Health Canada GCDWQ, Ontario Regulations 169/03 and 170/03, US EPA drinking-water rules where the team benchmarks across jurisdictions) and figuring out what each rule actually requires, where our SOPs match it, where they don't, and what changes when a regulation shifts. The regulatory change log I maintain alongside the team's consolidated SOP is change impact analysis in practice: when a federal or provincial rule shifts, I log the change, identify which SOPs and procedures are affected, surface the compliance risk to project leads, and coordinate the SOP revision that has to follow. Beyond rule text, I read peer-reviewed work on enzymatic β -glucuronidase detection and other rapid microbial monitoring methods to understand how alternatives to culture-based lab testing have been validated, and I use that to frame our pilot analysis of ColiMinder outputs against culture-based references.

I also build Power BI heat maps of microbial, chemical, lead, and nitrate exceedances across Canadian and US jurisdictions using public Health Canada, US EPA, and provincial reports, which the team uses for cross-jurisdictional comparison. When pilot data comes in, I run ColiMinder real-time outputs against culture-based lab references in Excel, calculate percent agreement across sites, surface where alarm signals diverge from confirmed lab outcomes, and translate the result into comparison tables and briefs project leads use to decide tradeoffs in speed, verification, and regulatory acceptance. Same pattern as the Clinical Research Assistant role: extract from primary sources (regulatory and peer-reviewed), compare against reference, structure the comparison so a decision-maker can act.

At IRCC's Operations Support Centre, the work is investigative. When a biometric system failure arrives from one of 60+ overseas Visa Application Centres or Missions, the question isn't what failed but where in the chain (submission, transmission routing, downstream IRCC partner system response) and what the right resolution path is under IRPA. That requires reading the trace, interpreting partner-system responses, and documenting reasoning so the next person on the file has full context. Same investigative habit applies to identity discrepancies: UCI duplicates, biometric re-associations, unlinked identifiers that block application processing. Nothing gets resolved by skimming.

My BSc in Biology from the University of Ottawa is the foundation. Four years of microbiology coursework with structured sample handling, statistical analysis of lab datasets in Excel, and technical lab reports laying out methods, results, and limitations. That training is what makes me comfortable with medical and scientific language, research data, and the structured-skepticism mindset systematic reviews and meta-analyses require. I haven't done formal systematic review work yet, and I'd be honest about wanting to learn that from senior team members here. The underlying habits are already there.

Outside of work, I've been building a self-directed project (curious-brain) that pulls academic papers from arXiv and Semantic Scholar nightly on configured topics, screens them, and synthesizes structured summaries. It's a small but practical version of the literature-screening-to-structured-output pipeline this role runs at scale, and it's kept me close to primary research outside my formal commitments.

I'm bilingual in English and Arabic with working knowledge of French, eligible for federal security clearance, currently managing two simultaneous roles, and genuinely looking for the chance to bring this combination of primary-source reading, structured data work, federal context, and a builder's curiosity to evidence-based health technology assessment.

Thank you for your consideration.

Sincerely,

Husam Ahmed